MIXED PROTOCOL STUDY: Intravascular Lithotripsy for Severely Calcified Coronary A

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The evaluation of IVL requires multimodal imaging.
Patients undergoing interventional revascularization for Left Main (LM) or CTO disease exhibited superior clinical outcomes when IVUS guidance was documented.

SCANNED ATTACHMENT: Angiographic Run #6

IVUS MLA: 3.8 mm² | IVL Pulses: 80 | Stent: 3.5 x 28 mm

Residual In-Stent Restenosis (ISR): None.